

INVESTMENT PROPOSAL

Janta Telehealth

Decentralized Micro-Clinics and D2D2P light asset telehealth
SaaS addressing healthcare deficits across rural India for
people and livestock

Ideator: Rahul Patel

IITR Graduate (2005) & Grassroots Organizer

The Founder



IIT Pedigree + Deep Local Trust

Rahul Patel (IITR Graduate, 2005) combines 10 years of full-stack software development across multiple leadership roles, 5 years of hands-on operational entrepreneurship with a decade of grassroots leadership.

- ✓ **Medical Camp Executor:** Successfully organized 27 medical camps over 3.5 years across Rohtas villages (Bihar)
- ✓ **Vast Village Network:** District Secretary of Kisan Mahasangh & active member of Gramin Chikitsak Sangh, Bihar

The Healthcare Gap (Bihar, India)

 **Extreme Doctors Scarcity:** acute shortage of certified doctors overall and 80% of population nearly unrepresented

 **The Travel Surcharge:** Loss of 1-4 workdays, burden of cost to food, commute and sometimes for stay & inconveniences just for OPD vis-a-vis falling earning and its opportunity

 **Delayed or Misdiagnosis:** Problem of overdose, misdiagnosis on hands of RMPs, lack of timely intervention and delayed decision to visit a doctor turning costly

 **Catastrophic veterinary gap:** Barely functioning veterinary care leads to severe livestock loss and under productivity

Critical Care Ratios in Bihar

Doctor : People

1:2702 (1:1000 WHO)

Doctor : Cattle Unit

1:10312 (1:5000 WOAH)

Of total doctor visit, 90% is OPD

✓ 8% visit Government Hospitals

✓ **23% visit Private Doctors**

✓ **56% visit RMPs**



The Solution

Instant Digital Connectivity: Light asset, low-bandwidth telehealth SaaS providing video/audio teleconsultations by doctors to rural patients in D2D and D2P model and EHR facility

Assisted E-Clinics: Leveraging vast network of RMPs as Facilitator in D2D model by using their treatment protocol awareness & upgrading their infrastructure for vastly non-tech-savvy and/or low trusting patients

Accessible Medical Needs: Medicine, Pathology tests, remote monitoring facility, 1-2 bed for IPD available at RMPs' facility

Key Benefits to the Ecosystem:

- 25% Cost Reduction compared to city visits, no loss to work & inconvenience
- 24/7 Access to verified doctors from anywhere, switching models
- Integrated facilities of doctor, medicine, pathology tests & IPD
- EHR to keep patients records together and accessible



Market Opportunity (Bihar, India)

\$22.8M

Annual Commission
Pool in Bihar alone

20.75%

YoY Digital Health Growth

3:1

Preference to Pvt vs
Government Facilities

90%

Workers earning less
than \$100 monthly

133.4K

Daily Pvt OPD, 58M will
prefer Telehealth by 2035

27.5M

Cattle Units in need of
Veterinary Doctors

- There are other BIMAROU states in India with **similar gaps and TAM**
- Even States with good doctor : population ratio **suffer same rural-urban divide**
- Despite ~100% healthcare density, **nearly all telehealth companies** got traction in urban centers only owing to traffic jams, long waiting time and inconvenience
- Public initiatives like eSanjeevani, falling earning and inconvenience awareness are creating **perfect tailwind** for rural, accessible telehealth infrastructure.

Transactional Unit Economics (The Model)

Operational Model	Standard Fee Structure	Revenue Distribution	Primary Benefit
Doctor to Facilitator (D2D)	10% of consulting fee or min 100/-	1:1 to Facilitator & Platform	Bridges internet accessibility & trust deficit
Doctor to Patient (D2P)	5% of consulting fee or min 50/-	100% platform retained	Perfect for urban and out of station follow-ups
Value Added Channels	Direct markups or commission on drugs and labs	Platform owned or share with third party contractor	Ecosystem Monetization

Operational Strategies (The Model)



Fallback Protocols

Automatic fallback transitions from real time high-def video to basic phone calls in case of low or no bandwidth



Organic Referrals

Facilitators onboard their city doctors to the platform in D2D, generating localized networks with zero acquisition cost



Trust-Centric Escrow

Consultation fees are held in virtual escrow and available to withdraw only upon successful consult completion



Diagnostic Camps

Deploying the Founder's verified regional medical camp format to build local awareness

Our D2D2P Gassroots SaaS

95% (High Local Trust & Light Asset Capex)

Franchise Teleclinics (Apollo)

60% (High Capex, Slow Deployment)

Standard D2P Apps (Practo)

45% (Fails on Rural Trust)

Govt eSanjeevani

30% (Severe Trust & Supply Deficit)

While traditional competitors suffer from either high asset-heavy operational overhead (franchise systems) or lack of local trust (direct-to-consumer apps), our model utilizes trusted local practitioners as patient interpreters.

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Services Offered (The Model)

Notification Service

Triage Service

Payment Service

**Waiting Room
Service**

**Telemedicine
Service**

**Healthcare
Education**

**Telemonitoring
Service**

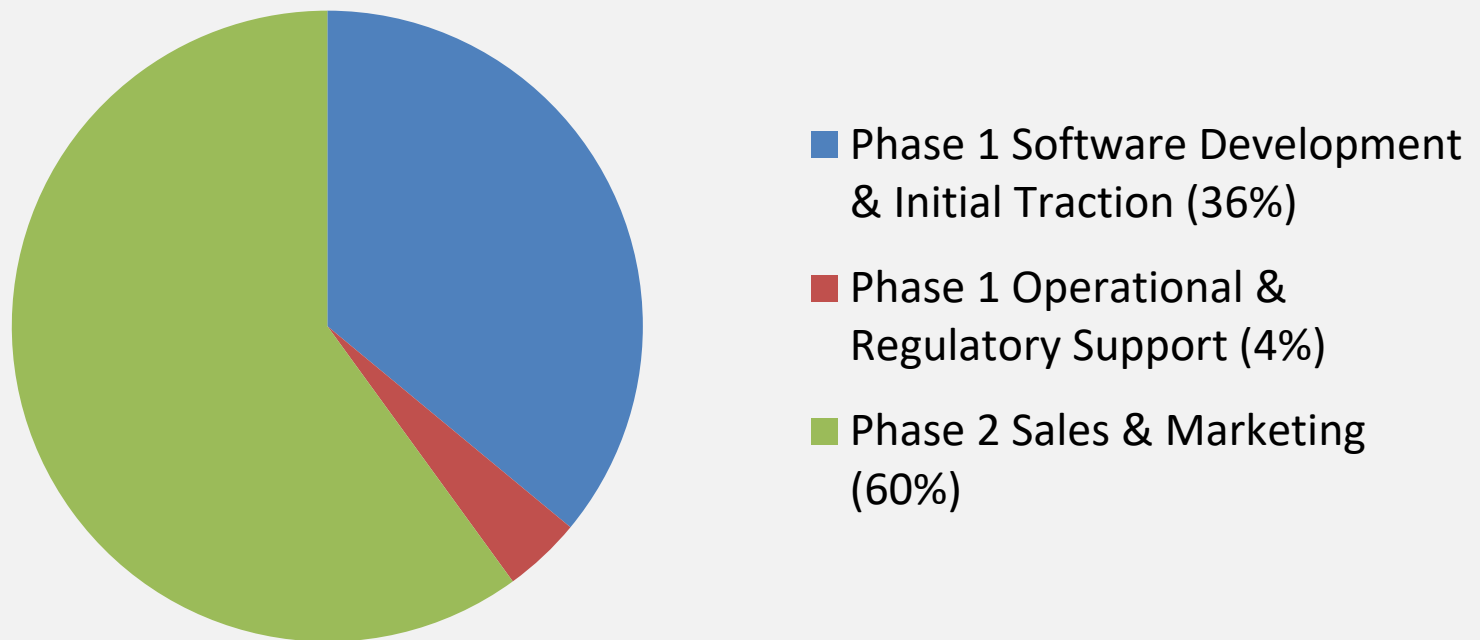
**Document Editing
Service**

EHR Service

Multilanguage Support

The Ask

Capital Deployment



Seeking **₹ 10.0 Crore Private Equity for 15% stake** into milestone-based 2 tranches to build the MVP and then establish rural networks in Bihar and other states across India

Questions & Answers

Join us in transforming decentralized healthcare access for rural communities



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